

AI-Powered HMS Synthetic Clinical Dataset

Clinical Note

Patient Name	Truong Minh Quan	MRN	MRN-0043
DOB	1951-10-18	Gender	Female
Encounter Date	2025-01-01	Department	Neurology
Attending Provider	BS. Nguyen Thi Lan	Status	active

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Truong Minh Quan is a 74-year-old patient in Neurology presenting for follow-up of episodic neurologic symptoms with need for monitoring. PMH includes episodic headache or transient weakness, no persistent focal deficit on exam. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include levetiracetam 500mg BID; atorvastatin 20mg nightly; aspirin 81mg daily. Allergies: latex sensitivity.

Objective

Vitals: BP 118/83 mmHg, HR 65 bpm, RR 17/min, Temp 36.5 C, SpO2 93% RA, weight 56 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-05-03	Creatinine	1.31	mg/dL	0.7-1.3	High
2026-05-03	eGFR	60.0	mL/min/1.73m2	>=60	Normal
2026-05-03	Glucose	100.0	mg/dL	70-99	High
2026-05-03	HbA1c	6.1	%	<5.7	High
2026-05-03	BNP	320.0	pg/mL	<100	High
2026-05-03	AST	28.0	U/L	10-40	Normal
2026-05-03	ALT	33.0	U/L	7-56	Normal
2026-05-03	Potassium	4.3	mmol/L	3.5-5.0	Normal
2026-05-03	Hemoglobin	13.9	g/dL	12.0-17.5	Normal

Assessment

Primary assessment: transient neurologic symptoms, seizure history, or migraine syndrome under evaluation. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Document neuro checks and fall precautions. Review MRI/CT results when available. Escalate for new weakness, aphasia, seizure, or GCS decline. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. transient neurologic symptoms, seizure history, or migraine syndrome under evaluation. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Neurology.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: clinical_note.